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- (b) \$42,900.00 for future care costs;
 - (c) \$7,550.00 for past loss of housekeeping and home maintenance;
 - (d) \$39,000.00 for future costs of housekeeping and home maintenance;
 - (e) \$10,500.00 for past loss of income;
 - (f) \$12,400.00 for future loss of income
- \$212,350.00

In addition, counsel had agreed that the defendants would pay the additional sum of \$2,932.19 for the plaintiff's chiropractic fees not covered by insurance if she was successful on the threshold issue.

- [3] This action, in terms of the plaintiff's injuries, is predicated on her chronic pain condition constituting "a permanent, serious impairment of an important physical, mental or psychological function".
- [4] The previous "threshold" provisions set out in the former s. 266, *Insurance Act*, R.S.O. 1990 c. I.8 was interpreted by the Court of Appeal in *Meyer v. Bright*, 15 O.R. (3d) 129, wherein the court concluded that the appropriate approach in such cases was to sequentially answer the following question:
 - 1. Has the injured person sustained permanent impairment of a bodily function caused by continuing injury which is physical in nature?
 - 2. If the answer to question number 1 is yes, is the bodily function, which is permanently impaired, an important one?
 - 3. If the answer to question number 2 is yes, is the impairment of the important bodily function serious?

Notwithstanding the court's admonishment of the use of the term "threshold", as "inaccurate", it continues to date in common parlance.

- [5] One of the three actions under consideration in *Meyer v. Bright* was that brought by the plaintiff Anne Dalglish who had sustained soft tissue injuries to her neck and low back. It was conceded that her injuries impaired her bodily functions, that they were important to her and that her impairments were permanent. The issue was whether the impairment was serious. However, the decision of the motion judge was based only on materials. At paragraph 94, the appellate court remarked:

94 His decision was based entirely upon documentary material composed of the transcript of Mrs. Dalglish's examination for discovery, affidavits and medical reports. His decision was not based

upon his assessment of the credibility of witnesses. Therefore, we are in as good a position as was he to decide the issue. It is not necessary to accord the deference to his decision to which it would be entitled if it were a decision rendered after trial.

GOVERNING LEGISLATION

[6] The July 9, 2012 accident is governed by Regulation 381/03, Bill 198. Section 267.5(5) of the *Insurance Act* relieves the owner of an automobile from liability for non-pecuniary losses unless the injured person has sustained “a permanent, serious impairment of an important physical, mental or psychological function”.

[7] Section 267.5(5) states:

(5) Despite any other Act and subject to subsection (6), the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for non-pecuniary loss, including damages for non-pecuniary loss under clause 61(2)(d) of the *Family Law Act*, from bodily injury or death arising directly or indirectly from the use or operation of the automobile, unless as a result of the use or operation of the automobile the injured person has died or has sustained,

permanent serious disfigurement; or

permanent serious impairment of an important physical, mental or psychological function.

This provision has application to the plaintiff’s recovery of general damages for pain and suffering and loss of enjoyment of life.

[8] Section 267.5(3) states:

(3) Despite any other Act and subject to subsections (6) and (6.1), if the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for expenses that have been incurred or will be incurred for health care resulting from bodily injury arising directly or indirectly from the use or operation of the automobile unless, as a result of the use or operation of the automobile, the injured person has died or has sustained,

(a) permanent serious disfigurement; or

(b) permanent serious impairment of an important physical, mental or psychological function. 2002, c. 22, 2. 120(1); 2011, c. 9, Sched. 21, 2. 3(2).

This provision has application to the plaintiff’s recovery of Cost of Future Care.

[9] The Regulation provides definitions of the criteria required to be met to enable the plaintiff to satisfy the “threshold”. The three criteria are numbered as subparagraphs 1, 2 and 3 of s. 4.2(1).

[10] Section 4.2(1)1, the impairment must,

- (i) substantially interfere with the person’s ability to continue his or her regular or usual employment, despite reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue employment,
- (ii) substantially interfere with the person’s ability to continue training for a career in a field in which the person was being trained before the incident, despite reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue his or her career training, or
- (iii) substantially interfere with most of the usual activities of daily living, considering the person’s age

but only one of the three subparagraphs need be met to satisfy the criterion of a serious impairment.

[11] Section 4.2(1) 2,

- 2. For the function that is impaired to be an important function of the impaired person, the function must,
 - (i) be necessary to perform the activities that are essential tasks of the person’s regular or unusual employment, taking into account reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue employment,
 - (ii) be necessary to perform the activities that are essential tasks of the person’s training for a career in a field in which the person was being trained before the incident, taking into account reasonable efforts to accommodate the person’s impairment and the person’s reasonable efforts to use the accommodation to allow the person to continue his or her career training,
 - (iii) be necessary for the person to provide for his or her own care or wellbeing, or,
 - (iv) be important to the usual activities of daily living, considering the person’s age.

but only one of the four paragraphs need be met to satisfy the criterion of an important function.

[12] Section 4.2(1) 3,

3. For the impairment to be permanent, the impairment must,
 - (i) have been continuous since the incident and must, based on medical evidence and subject to the person reasonably participating in the recommended treatment of the impairment, be expected not to substantially improve,
 - (ii) continue to meet the criteria in paragraph 1, and
 - (iii) be of a nature that is expected to continue without substantial improvement when sustained by persons in similar circumstances.

All aspects of these three paragraphs must be satisfied for the plaintiff to have met this criterion.

[13] Section 4.3 of the Regulation sets forth the nature and extent of medical evidence to be adduced to support the plaintiff's claim that she satisfies the "threshold", as well as evidence to corroborate the change in the function alleged to meet the "threshold" requirements.

[14] Similar to the test established in *Meyer v. Bright*, a sequential assessment of the following three questions is required to conclude whether the plaintiff has met the requirements of s. 267.5(5)(b):

1. Has the injured person sustained permanent impairment of a physical, mental or psychological function?
2. If yes, is the function which is permanently impaired an important one?
3. If yes, is the impairment of the important function serious?

POSITION OF THE DEFENDANTS

[15] The defendants have adopted the position that the plaintiff does not meet any of the three criteria essential to satisfying the "threshold" requirements. On the requirement of permanence, the defence theory is based on injury sustained by the plaintiff in a subsequent motor vehicle accident of June 14, 2014 as the *causa causans* of her continuing current symptoms and that she has failed to prove on a balance of probabilities that her condition is attributable to the initial accident of July 9, 2012.

[16] In respect to refuting the criterion for importance, the defence rely on the fact that the plaintiff continued to work at her part-time employment for almost 4.5 years post-

accident, earning marginally more in certain years, suggestive of increased hours. Further, that the plaintiff performed a significant amount of her housekeeping duties. Lastly, that she completed an accident benefits form (OCT-12) titled Activities of Normal Life in conjunction with the June 14, 2014 accident which indicates that prior to the June 14, 2014 accident, the plaintiff was independent in activities of daily living.

- [17] Concerning the requirement that the impairment be serious, the defendants rely on the factors set out in the two proceeding paragraphs as well as the fact that her accident benefit claims regarding both accidents were processed in accordance with minor injury guidelines and that designation was never contested.

POSITION OF THE PLAINTIFF

- [18] Plaintiff's counsel contends that the plaintiff satisfies all three criteria of s. 267.5(5)(b) of the *Insurance Act*, R.S.O. 1990, c.I.8, namely, injury sustained in an automobile accident that constitutes "a permanent serious impairment of an important physical, mental or psychological function", as defined therein. It is her position that she meets all three criteria on the basis of her forced early retirement alone as well as the impact on her usual activities of daily living and not expected to substantially improve.

BACKGROUND

- [19] The plaintiff, born September 19, 1950, was a front-seat passenger in her vehicle, being driven by her daughter, on July 9, 2012 at approximately 6:00 p.m. on a dry, clear and sunny afternoon when her vehicle was struck from the rear while stopped at an intersection by one of the defendants who was charged and paid a fine. Both vehicles sustained damage. The plaintiff was taken to hospital very shortly following the accident, examined, x-rayed, and released with medication and instructions to follow up with her family doctor. Her complaints were of pain in her neck, shoulders and down her back, and headaches. The diagnostic imaging report was negative as to any fracture.
- [20] Although the plaintiff had pre-existing health conditions including diabetes, hypertension and hypothyroidism, they were controlled and were unrelated to injury sustained in this accident. She had no previous history of neck, shoulder, back or headache complaint or treatment. Her family doctor provided a note for her to remain off work for six weeks and attend for physiotherapy which she did. She returned to work at Sears in only four weeks on modified physical restrictions and was discharged from physiotherapy October 12, 2012. The plaintiff readily acknowledged that she was improving and her symptoms were getting better.
- [21] Unfortunately, she had a set-back in early January 2013. She awoke in agony one morning without having sustained any intervening trauma or personal injury. Unable to get a prompt appointment with her previous physiotherapist, she attended Dr.

Robert Campana, D.C. C.Ped.(C), Dr. of Chiropractic, for an initial examination, conducted January 9 and 10, 2013.

[22] Dr. Campana's diagnosis was:

Fibromyalgia;
Subluxation complex (vertebral), cervical region;
Whiplash associated disorder [WAD2] with complaint of neck pain with muscul;
Subluxation complex (vertebral), thoracic region;
Subluxation complex (vertebral), lumbar region;
Sprain and strain of sacroiliac joint

The plaintiff has consistently continued to treat with Dr. Campana from January 9, 2013 to the date of trial, February 5, 2018.

INDEPENDENT MEDICAL ASSESSMENTS

Dr. Watson

[23] Plaintiff's counsel scheduled a consultation for the plaintiff with Dr. C. Peter H. Watson, M.D., F.R.C.P.C., Neurology. His report dated May 21, 2014 was entered as a trial exhibit and he testified. He has been in neurological practice since 1973 and has published extensively in the field of neurology and chronic pain. He is Associate Editor of the journal "*Pain Research and Management*". From 2005-2013 he was a Tribunals Assessor, Chronic Pain and Neurology, with Worker Safety and Insurance Board (WSIB).

[24] The plaintiff attended Dr. Watson on May 15, 2014 and informed him that she has had neck pain, shoulder pain, and low back pain since the automobile accident of July 9, 2012. Based on her reported history, his physical examination and review of her medical records, including those of the family doctor, the physiotherapist, and Dr. Campana, he expressed the following opinion:

Ms. Mousseau has had chronic neck, shoulder and low back pain since the accident of July 9, 2012 (almost 2 years ago). The pain goes down her arms suggestive of a neuropathic pain component. The pain is moderate to severe in intensity and has a significant impact on her life. I think the prognosis at this time can be reasonably viewed as guarded since it has been nearly two years since the accident.

[25] In response to specific questions put to him by plaintiff's counsel, he responded:

- (a) The diagnosis is post-traumatic neck, shoulder and low back pain. The prognosis is guarded;
- (b) She would be best managed by a chronic pain expert or in a pain clinic;

- (c) Her constant moderate to severe pain is a permanent serious impairment due to injuries sustained in the accident of July 9, 2012;
- (d) She is no longer able to lift heavier objects more than 20 lbs, does not do any overhead work, and her hours are limited to about 20 hours a week so there has been an impact on her work and it is reasonable to state that her longevity of working career has been curtailed.

Dr. Watson confirmed at trial that chronic pain is a recognized medical issue.

Dr. Berbrayer

- [26] Defence counsel scheduled a consultation for the plaintiff with Dr. David Berbrayer, M.D., BSc, (MED), FRCPC, DABPMR, FAAPMR, Physiatrist. His report thereon, dated July 25, 2015 was entered as an exhibit at trial and he also testified. He is a specialist in physical medicine and rehabilitation and has practiced since 1976. He is currently Chief of Physiatry at Sunnybrook Health Service Centre. Dr. Berbrayer reviewed her history, conducted a physical examination and reviewed her medical records.
- [27] Dr. Berbrayer determined the plaintiff to be a very credible individual; easy to interview; cooperative throughout; and who addressed all his questions. His diagnosis was that she suffered uncomplicated soft tissue injuries of the neck and back from the accident of July 9, 2012. The plaintiff disclosed the accident of June 14, 2014 in which he diagnosed her to have sustained a re-exacerbation of myofascial pain of the cervical spine, mechanical low back pain without neurological deficit and cervicogenic headaches.
- [28] He believed her overall prognosis to be stable and that she continues to have neck and mid-back pain. He noted that she improved after the first accident. Dr. Berbrayer was of the opinion that her neck and back pain had disappeared by the time of the second accident but returned after the second accident. She continues to have neck and mid-back pain and mild low-grade headaches since the second accident.
- [29] In response to specific questions put to him by defence counsel, Dr. Berbrayer stated that the plaintiff received appropriate treatment; that she has reached maximum medical recovery regarding the first accident; the symptoms from the two accidents are identical. He concluded by stating that the plaintiff has not sustained a permanent and serious injury or impairment which interferes with important bodily functions as a result of the motor vehicle accident of July 9, 2012, and that she has returned to her former employment. He goes on to essentially disqualify the plaintiff from satisfying or fulfilling any of the definitions set out in the Regulation.
- [30] Nonetheless, Dr. Berbrayer concludes his report with the following opinion:

As a practicing physiatrist, I am qualified by education, training, and experience to express the opinion that the injuries and impairments of the Examinee are serious and permanent. This concludes my report.

At the conclusion of his evidence in-chief, he was asked if there was anything he disagreed with in his report and he answered, no, that he stands by his report.

Dr. Campana

- [31] Dr. Campana testified at trial respecting the period over which he has been treating the plaintiff, essentially to date, since January 9, 2013. His curriculum vitae was entered as an exhibit at trial. Dr. Campana began his practice in 1981 and has extensively treated a large number of patients suffering from soft tissue injuries sustained in motor vehicle accidents. He informed that 60 to 70 percent of patients recover; one-half get better after one year; and 10 to 15 percent become chronic.
- [32] He testified that the plaintiff has chronic pain and is “not one to complain much”. Her injuries and symptoms from the July 9, 2012 accident have not resolved and when she does complain there is evidence in his examination to confirm it. She accommodates herself in terms of activities of daily living so as not to bring on pain. He was led through his extensive clinical notes by both counsel and which demonstrated a fluctuation in her degree of pain. At times she is doing better, while at others she is worse. In relation to his diagnosis of fibromyalgia, amongst others, he elicited muscle pain on several quadrants of her body. She had 16 of the 18 trigger points. It is caused by trauma, physical or emotional, and doesn’t usually resolve. He considered there to be subtle differences between chronic pain and fibromyalgia and that they are interchangeable terms.
- [33] Upon his cross-examination, Dr. Campana affirmed that her condition is chronic and will be there forever. She will never be 100 percent better but she has improved from when first seen with pain rated at 9 of 10, with 10 being the worst. On the implications of the second accident, June 14, 2014, Dr. Campana testified that the latter didn’t cause nearly as much injury as the former, and proffered that he probably wouldn’t be treating her solely for injury in the second accident were it not for the nature and extent of her physical injury sustained in the first accident.

Co-Workers

- [34] Two longstanding fellow employees, one a manager, of the plaintiff at Sears, her place of continuous part-time employment since 1994, testified. Ramona Dodd was a co-worker/manager for over 20 years at Sears. She testified that she was acquainted with the plaintiff in both of the witness’s capacities until the plaintiff’s retirement October 29, 2016. The plaintiff very much enjoyed her job. Prior to July 9, 2012, the plaintiff was physically unrestricted in her duties. She was efficient, punctual and did her job without ever complaining of neck or back pain. The plaintiff had expressed her intention to work longer.

- [35] After the accident of July 9, 2012, the plaintiff was off for a while and returned to her previous department with modified hours and duties and physical restrictions in terms of lifting, pushing and pulling goods and displays. The plaintiff eventually resumed her former part-time hours but continued on an accommodated basis with restrictions that increased with time up to her retirement. Her fellow employees assisted her in performing what was expected of her. The plaintiff complained of increasing pain from time to time in her neck, shoulders and back.
- [36] Suzanne Smidt, 62 years of age, testified in relation to the plaintiff's employment circumstances. Ms. Smidt was a fellow employee who worked in the hardware department with the plaintiff. The witness had been at Sears for 37 years and knew the plaintiff through work for 20 years. In reference to the plaintiff prior to her July 9, 2012 accident, Ms. Smidt described the plaintiff as dependable; reliable; one who did her share and more; hard worker; never booked off shifts; handled her duties with ease and without limitations. Following her return to work post-accident, the plaintiff was accommodated with afternoon as opposed to morning hours. She was significantly restricted in her ability and expectation to lift; stretching; climbing; and twisting. Her pain was in the neck, shoulder and back areas. She would take medication for it at times. Her fellow employees all "stepped up" to help her fulfill her employment duties. Prior to the accident there wasn't discussion about how long the plaintiff would continue to work but afterwards there was. From a financial perspective, the plaintiff expressed that she wanted and intended to keep working to meet financial obligations. The pain became too much of "a grind" for her and she retired October 29, 2016. Her situation had never improved and probably became worse in her last year or so.

Spouse

- [37] The plaintiff's husband of 49-plus years testified at trial. He is 68 years of age and continues to work on a full-time basis to meet their financial obligations. He advised that his wife did the vast majority of the indoor homemaking tasks while he took care of the outdoor chores. He confirmed that his wife enjoyed her part-time job and her fellow employees at Sears and would have continued to work but for the effect of injury sustained in the July 9, 2012 accident. She did not have any previous neck, shoulder or back issues before that accident but which continue to date. He attended at the accident scene within five minutes thereof and before the vehicles were moved. In 16 years of experience working a car lot, he concluded that the automobiles would have "hit hard". Although he was away at the time of the second accident, he examined the vehicles and concluded that the impact was a "slight hit" without hard jarring. His wife continued to work on an accommodated basis with physical restrictions in her duties but her condition worsened to the point that she was coming home "hurting" and decided to retire prematurely. He confirmed that his wife was no longer capable of accompanying him in walking their dogs around Whitefish Island, a distance of some four to four and a half kilometres, which they did "especially on weekends" during their leisure time. He finds himself having to perform the heavier,

more onerous homemaking tasks now and involving an average of two hours a day in addition to his full-time job.

JURY VERDICT

- [38] The award of the jury was set out in paragraph two above, and on a motion of this nature, I am permitted to consider it “and particularly where the symptoms are subjective”, *Kasap v. MacCullum*, [2001] O.J. No. 1719, at para. 7. The jury verdict is but a factor in the exercising of my judicial discretion in deciding whether or not the threshold has been met, and I am not bound by the jury’s apparent findings of credibility. However, I am not to be blinded to them either.
- [39] In any event, I too, as did Dr. Berbrayer, find the plaintiff to have been a refreshingly credible and reliable witness, as was her husband, and also the evidence spoken by the plaintiff’s fellow employees, Ms. Dodd and Ms. Smidt. In this case, having heard their *viva voce* evidence in addition to their reports and enumerated backgrounds regarding their evaluation of the condition of this particular plaintiff, I do prefer the evidence of Dr. Watson, as supported through the testimony of Dr. Campana, on the critical aspects for my adjudication.

RULING

Permanent Impairment (s. 4.2(1)3)

- [40] Approximately 68 months have elapsed since the accident of July 9, 2012 and the plaintiff continues to suffer from chronic pain in her neck, shoulders and back attributable to the injury sustained in the accident of July 9, 2012. Every indication based on all of the evidence except for Dr. Berbrayer, is that her chronic pain will last indefinitely into the future as opposed to any suggestion of a specific timeframe with a defined termination. Dr. Berbrayer’s opinion was that the plaintiff’s symptoms had completely resolved prior to the second accident. I find as a fact, as the jury must have, based on the amounts awarded on all heads of damage claimed by the plaintiff, that the plaintiff’s symptoms and chronic pain had not completely resolved considering all of the evidence. The plaintiff followed any recommended treatment, continues to medicate and to be treated by Dr. Campana. Dr. Watson stated that “her constant, moderate to severe pain is a permanent serious impairment at this time due to injuries sustained in the accident”. I am satisfied that she has not recovered to her pre-accident condition of no limitation in function in the areas injured in the accident. I find that her impairment is permanent.

Important Function (s. 4.2(1)2(i))

- [41] While the plaintiff’s permanently impaired function has negatively affected her entire way of life in terms of activities of daily living and leisure past times, it is the effect on her former employment that is of greatest importance. As a result of injury sustained in the July 9, 2012 accident she was never able to perform the activities that were essential tasks of her regular or usual employment. She returned to work as an

accommodated worker with physical restrictions and limitations and she complied therewith to enable her to continue with her modified duties for almost 4.5 years. However, her limitations grew greater and she increasingly relied on the support and assistance of her coworkers to function in her employment environment.

- [42] Notwithstanding her perseverance, she came to realize that a premature retirement was inevitable, retiring two years sooner than anticipated. The plaintiff was a person who enjoyed her work and enjoyed working. She enjoyed her relationships with fellow employees. She was responsible and dedicated and needed the income that employment provided to her and which her husband expected would continue, particularly in consideration of their combined financial obligations and requirements for retirement. I find that the function which is permanently impaired is an important one.

That is Serious (2. 4.2(1)(1)(i))

- [43] The impairment is the focus as opposed to the injury itself when considering the criterion of “serious”. The significance of the effect of injury on one’s employability was recognized in *Meyer v. Bright*,

34 It is simply not possible to provide an absolute formula which will guide the court in all cases in determining what is “serious”. This issue will have to be resolved on a case-to-case basis. However, generally speaking, a serious impairment is one which causes substantial interference with the ability of the injured person to perform his or her usual daily activities or to continue his or her regular employment.

- [44] In this case, the plaintiff became unable to continue with her accommodated employment due to injury sustained in the motor vehicle accident of July 9, 2012. Defence counsel submits that the fact the plaintiff continued to work for almost 4.5 years following the injury sustained in the accident, in itself demonstrates that her impairment was not “serious” as contemplated in Bill 198. Fortunately, for the plaintiff, the jurisprudence sees it otherwise as an issue of fact and as case specific.

- [45] Dr. Watson was of the opinion that the plaintiff’s physical restrictions had an impact on her work and that it was reasonable to state that her longevity of working career has been curtailed. I find that the plaintiff’s efforts to utilize the accommodation so that she could continue to work in chronic pain were beyond reasonable. Her husband testified that she would come home from work “hurting”.

- [46] In *Brak v. Walsh*, 2008 ONCA 221, the court held at paras. 21 and 22,

21 For some plaintiffs with chronic pain, it may be possible for them to do all of the things that they did before the injury on an occasional basis. The requirement that the impairment be “serious” may be satisfied even though, through determination, the plaintiff resumes the activities of employment or the responsibilities of the household but continues to experience pain.

22 A return to work, even where a person actually earns more money, will not necessarily result in a finding that the plaintiff does not have a permanent and serious impairment of an important physical, mental or psychological function. The plaintiff's ability to return to work and continue his or her employment is not fatal to a threshold motion. The courts look at the totality of the evidence, including the impact of the impairment on the plaintiff's ability to continue working and the impact of continued work on the plaintiff.

[47] I find that the plaintiff continued to work over the ensuing approximate 4.5 years through her perseverance, fortitude and sheer determination, in furtherance of her admirable work ethic, and that the impairment of the important function is indeed serious.

CONCLUSION

[48] I find that the plaintiff has established a permanent, serious impairment of an important physical, mental or psychological function resulting from the motor vehicle accident of July 9, 2012. Accordingly, the defence motion is dismissed. The plaintiff is entitled to judgment pursuant to the jury's verdict. If costs cannot be agreed upon, they are to be addressed upon an appointment before me as scheduled through the trial coordinator.

McMillan J.

Released: February 28, 2018

CITATION: Mousseau v. Morrison, 2018 ONSC 1274

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

DIANA MOUSSEAU

- AND -

AMANDA MORRISON and WILLIAM MORRISON

REASONS ON THRESHOLD MOTION

McMillan J.

Released: February 28, 2018